

Scheduling & Coordinating PT Referrals

Ancillary Services Coordination · Workers' Compensation

2 Hours

NEW ASSOCIATES · SOP → EXPERIENTIAL

THE SHIFT & WHAT THEY'LL LEARN

From rules to judgment. SOPs teach the rules; they don't build the judgment to apply them when a provider has no slot inside the SLA, a worker only speaks Spanish, or the auth is missing. Associates **do the real job on realistic cases**, get instant coaching, and reflect on *why*.

- ✓ Read a PT referral & spot what's needed (provider, appointment, transport, interpreter, auth)
- ✓ Schedule **within the jurisdiction SLA**, choosing the right provider under real constraints
- ✓ Run professional, empathetic calls — with the injured worker and the PT clinic
- ✓ Triage curveballs — missing auth, SLA risk, no-shows, cancellations: **solve vs. escalate**
- ✓ Document a clean, defensible coordination hand-off

THE 2-HOUR RUN-OF-SHOW

0:00 · 10	Open — from rules to judgment · framing + ground rules
0:10 · 15	Anatomy of a PT referral · DRILL spot the gaps
0:25 · 20	Role-play A — the PT clinic call · ROLE-PLAY
0:45 · 20	Provider selection under constraints · DRILL
1:05 · 5	Break
1:10 · 20	Role-play B — the injured-worker call · ROLE-PLAY
1:30 · 15	The curveballs — solve vs. escalate · DRILL
1:45 · 10	Reflection & synthesis · what changed
1:55 · 5	Wrap — one-case certification check · GATE

PLAYABLE SCENARIOS (DEMONSTRATE, THEN ROLE-PLAY)

1 Clean fast-track

2 SLA squeeze (TX 2-day)

3 Missing authorization

4 Transport + language

5 No-show / cancellation recovery

6 Clinical conflict → escalate

7 Out-of-network request

Certification gate: a fresh PT referral (TX, missing auth, Spanish-speaking, no transport) — associate writes the first 3 actions, the one thing to escalate, and the SLA deadline. Pass = SLA correct, schedules in-network toward it, surfaces auth + interpreter + transport, escalates only the right thing. **This is the gate before live work.**